Canberra's Emergency Department Wait Times: What 3 Years of Data Show

Nov 3, 2026 · @kyla skye

Summary

Canberra's emergency departments are getting slower, and Urgent patients wait the longest relative to the target. In 2025-26, only 58.6% of ACT emergency patients started treatment within the clinically recommended time, down from 62.0% two years earlier.

I used SQL to analyse 3,347 monthly records of official ACT Government data (July 2023 to June 2026). My question: where is Canberra's emergency care falling short, and is it improving?

- **Waits are rising.** The median wait to start treatment went from 25 minutes in 2023-24 to 28 minutes in 2025-26.
- **Urgent patients have the biggest gap.** 47.6% were seen within 30 minutes, against a national benchmark of 75%. Neither hospital met it in any of the 36 months.
- **Canberra Hospital is under the most pressure.** Its median wait was 30 minutes, compared with 23 at North Canberra Hospital.
- **Patients who need a bed get stuck.** 45% of admitted patients left the ED within 4 hours, compared with 66% of patients who went home.
- **Winter is the worst time.** Median waits average about 28 minutes in July and August, compared with 23 in January.

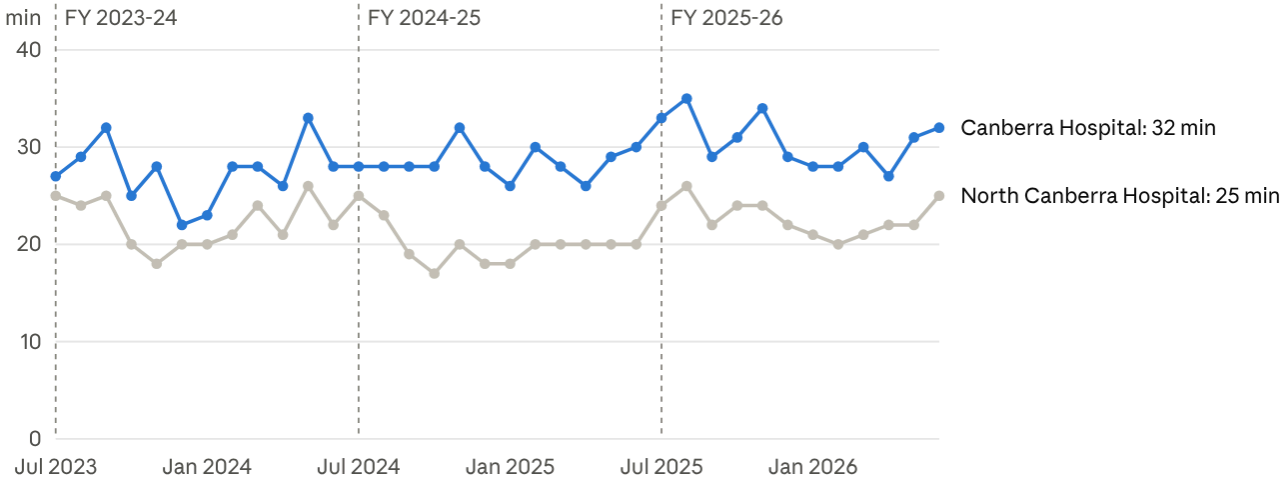
1. Waits got worse in 2025-26

2025-26 had the longest median wait and the lowest share of patients seen on time of the three years. The 4-hour measure improved in 2024-25, then slipped back.

Financial year	Median wait (min)	Seen on time (%)	Left ED within 4 hours (%)	Left without being seen (%)
2025-26	28	58.6	57.7	3.3
2024-25	26	61.6	60.6	2.6
2023-24	25	62.0	56.2	3.8

Median wait rose in 2025-26 and is longest at Canberra Hospital

Median minutes from arrival to start of treatment, all triage categories, by month



ACT Government Open Data Portal, ED performance data · monthly, Jul 2023 to Jun 2026

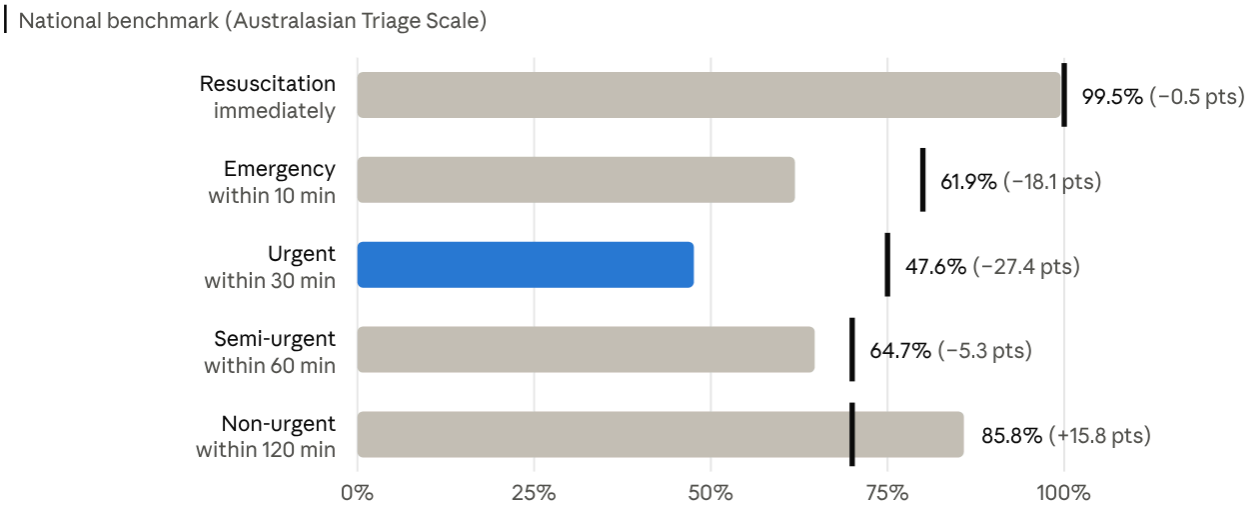
Both hospitals followed the same pattern, with a sharp rise in July and August 2025. Canberra Hospital's median wait was longer than North Canberra Hospital's in every one of the 36 months.

2. Urgent patients have the biggest gap

Every patient arriving at an ED is given a triage category, which sets how quickly they should be seen. In 2025-26 the ACT missed the national benchmark in four of the five categories. The gap was largest for Urgent (Category 3) patients.

Only 47.6% of Urgent patients were seen on time, against a 75% benchmark

ACT, 2025-26: % of patients who started treatment within the recommended time, by triage category



ACT Government Open Data Portal, ED performance data · ACT total, full year 2025-26 · benchmarks: Australasian Triage Scale

This is not a one-off. Across all 36 months, neither hospital met the Urgent benchmark even once. North Canberra Hospital never met the Emergency (Category 2) benchmark, and Canberra Hospital met it in only 4 months. Only Non-urgent patients were seen on time consistently.

3. Canberra Hospital is under the most pressure

Canberra Hospital is the territory's main hospital and major trauma centre. In 2025-26 it trailed North Canberra Hospital on almost every measure.

Measure, 2025-26	Canberra Hospital	North Canberra Hospital
Median wait to start treatment (min)	30	23
Seen on time (%)	55.0	64.4
Left ED within 4 hours (%)	56.0	60.3
Admitted patients leaving within 4 hours (%)	42.6	51.1
Left without being seen (%)	2.8	4.0

The exception is patients leaving without being seen, which was lower at Canberra Hospital.

4. Patients who need a bed get stuck in the ED

In 2025-26, only 45.4% of ACT patients admitted to a ward left the ED within 4 hours. For patients who went home, the figure was 66.2%. The gap has been about 20 percentage points in each of the three years. That suggests part of the delay comes from waiting for a free ward bed, not just from the ED itself.

5. Winter is the busiest time

Averaged across the three years, August had the longest median wait (28.3 minutes) and July was close behind (27.7). January had the shortest (23.3 minutes), when 65.8% of patients were seen on time, compared with 57.5% in August.

What this could mean for policy

- **Focus on Urgent patients.** Category 3 has the largest gap to benchmark, so improvements there would lift overall performance the most.
- **Look beyond the ED.** Admitted patients wait far longer to leave, so faster discharge and more ward capacity could ease ED crowding as much as extra ED staff.
- **Plan for winter early.** Waits rise every July and August, so surge staffing and beds need to be ready by June.

Limitations. This is monthly summary data, so it shows where problems are, not what causes them. It has no patient-level detail, such as age, diagnosis or time of day. Two monthly values for Resuscitation patients at North Canberra Hospital are missing. ACT Health also notes some figures may be affected by data-capture issues with its digital health record.

How I did it

I built the analysis in SQL (SQLite), with Python for loading the data and drawing charts.

1. **Loaded** the raw CSV, unchanged, into a staging table.
2. **Cleaned** it with SQL into a star schema: lookup tables for hospitals, measures and triage categories, plus one fact table. Text months such as "Jul 2023" became real dates, and blank values became NULL rather than 0.
3. **Added national benchmarks** from the Australasian Triage Scale, so actual performance could be compared with targets in a single JOIN.
4. **Answered 9 policy questions** with SQL, using aggregation, JOINs, subqueries, CTEs and window functions (RANK, LAG and rolling averages).
5. **Checked data quality** with a query that finds missing or blank months before trusting any result.

All the code, queries and results are on GitHub: github.com/kylaSkye/canberra-ed-wait-times

Sources

- ACT public hospital emergency department performance data, ACT Government Open Data Portal (data to June 2026)
- ACT Health: Emergency department performance dashboard